

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase II, Randomised, Double-Blind, Placebo-Controlled, Parallel Group Study to Assess the Effect of AZD0780 in Combination With Rosuvastatin on Low Density Lipoprotein Cholesterol in Participants With Dyslipidaemia (LAZURE)
Short Title:	A Study to Investigate the Effect of AZD0780 Tablets in Combination With Rosuvastatin Tablets on Low Density Lipoprotein Cholesterol Levels (LDL-C) in Adult Participants With Dyslipidaemia
Protocol Number:	227028409552753319
NCT Number:	NCT07218900
Sponsor Name:	AstraZeneca
Investigational Product:	rosuvastatin
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Relative change in Low-density lipoprotein cholesterol (LDL-C) from baseline to 12 weeks: To compare the effect of treatment with AZD0780 versus placebo on LDL-C at 12 weeks
Secondary Obj.:	1. Indicator for LDL-C < 70 mg/dL (< 1.8 mmol/L) at 12 weeks 2. Indicator for LDL-C < 55 mg/dL (< 1.4 mmol/L) at 12 weeks 3. Relative change in Apolipoprotein (Apo) B from baseline to 12 weeks

2.2 Background & Rationale

This is a study to investigate the effect of AZD0780 tablets on low density lipoprotein cholesterol levels (LDL-C) compared with placebo tablets administered in combination with rosuvastatin tablets in adult Russian participants with dyslipidaemia. AZD0780 is a small molecule that reduces the amount of LDL-C in the blood. Placebo will be used for comparison, and neither the participants nor the Investigators will know who is receiving the AZD0780 medication and who is receiving the placebo until the end of study.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: NOT_YET_RECRUITING
Target N:	76

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* ? 18 years of age at the time of signing the ICF

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* History of clinical Atherosclerotic cardiovascular disease (ASCVD) or at risk for a first ASCVD event:

1. Clinical ASCVD is defined as MI, stable or unstable angina, coronary or other arterial revascularisation, ischaemic stroke, or peripheral artery disease.

2. A participant is considered at risk for a first ASCVD event if the participant has one or more of the following conditions: atherosclerotic vascular disease (? 50% stenosis in ? 2 coronary artery territories or in ? 2 vascular beds [coronary, carotid, lower extremity], diagnosed by any imaging modality), diabetes mellitus, hypertension, cigarette smoking, chronic kidney disease (moderate to severe stage), or obesity. Investigators can also use the ACC/AHA or ESC or national clinical guidelines for risk assessment to identify participants with at least moderate risk for ASCVD.

* Fasting serum LDL-C by central laboratory at screening as follows: LDL-C ? 55 mg/dL (? 1.4 mmol/L) in participants with clinical ASCVD or ? 70 mg/dL (? 1.8 mmol/L) in participants without clinical ASCVD but at risk for a first ASCVD event

* Participants should meet 1 of the following before screening:

1. On a stable dose of Lipid-lowering therapy (LLTs) including moderate statins.
2. On a stable dose of LLTs without any statins (either have been previously treated or be statins treatment naïve).
3. Not received treatment with any LLTs (either have been previously treated or be LLTs treatment naïve).

4.2 Exclusion Criteria

* Homozygous familial hypercholesterolaemia, known diagnosis of heterozygous familial hypercholesterolaemia (HeFH), LDL apheresis or plasma apheresis within 12 months prior to screening, or any other underlying known disease or condition that may interfere with interpretation of the clinical study results as judged by the Investigator.

* Any of the following laboratory values at screening:

1. Calculated eGFR ≤ 15 mL/min/1.73 m²
2. AST or ALT $\geq 3 \times$ ULN
3. TBL $\geq 2 \times$ ULN (except for patients with Gilberts syndrome, where TBL $3 \times$ ULN is acceptable provided direct bilirubin $\leq 1.5 \times$ ULN)
4. Fasting triglycerides ≥ 400 mg/dL (? 4.52 mmol/L)
5. Creatine kinase $\geq 5 \times$ ULN
6. Urine albumin-to-creatinine ratio ≥ 500 mg/g

* Uncontrolled type 2 diabetes mellitus defined as HbA1C $\geq 9.5\%$ at screening

* Inadequately treated hypothyroidism defined as TSH ≥ 1.5 ULN at screening or participants whose thyroid replacement therapy was initiated or modified within the last 3 months prior to screening

* Use of mipomersen or lomitapide (cholesterol-lowering medications) within 12 months prior to screening or planned use during the study.

* Use of gemfibrozil within 1 week prior to screening or planned use during the study.

* Use of PCSK-9 inhibitors: evolocumab/alirocumab within 12 weeks of the screening visit or planned use during the study or inclisiran within 18 months of the screening visit or planned use during the study. Any other approved PCSK-9 inhibitor use within 5 half-lives prior to the screening visit or planned use during the study.

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11. Study Identifiers & Governance

Primary ID: 227028409552753319

Registry Number: NCT07218900

Sponsor: AstraZeneca

Study Title: A Study to Investigate the Effect of AZD0780 Tablets in Combination With Rosuvastatin Tablets on Low Density Lipoprotein Cholesterol Levels (LDL-C) in Adult Participants With Dyslipidaemia

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____